2.12 Electrolyte Imbalances*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Palpitations and muscle weakness **Objective** • Serum K^+ 2.9 mmol/L (or above 5.5 mmol/L) • Irregular pulse • ECG changes	**ACTUAL** Decreased activity tolerance related to muscle weakness and cardiac rhythm disturbance associated with potassium imbalance as evidenced by reported weakness, palpitations and ECG changes.	• Regular rhythm, serum K^+ 3.5–5.0 mmol/L and improved strength within 24 hours.	• Maintain continuous ECG monitoring as ordered. • Give potassium replacement, or potassium-lowering treatment, as prescribed. • Never give potassium as an IV push; use an infusion pump. • Repeat serum K^+ as ordered. • Assist with activity until strength returns.	• Met when rhythm and potassium are normal.
Subjective • Family notice new confusion **Objective** • Serum Na^+ 124 mmol/L • Disoriented to time and place	**ACTUAL** Acute confusion related to sodium imbalance as evidenced by disorientation and abnormal serum sodium.	• Oriented to time, place and person as sodium normalizes.	• Check orientation and consciousness every 4 hours. • Correct sodium slowly, as prescribed. • Restrict fluids if ordered. • Reorient her and keep surroundings calm.	• Achieved when oriented. • Seizures or deepening confusion are escalated.
Subjective *No specific complaint* **Objective** • Muscle weakness • Confusion • On diuretics	**RISK** Risk for adult falls as evidenced by risk factors: muscle weakness, confusion and medications.	• No falls.	• Put fall precautions in place. • Assist with all transfers. • Keep the call bell within reach.	• Outcome reached if she does not fall.

*References for this care plan: [35], [37], [48], [108], [118]. Full entries are listed in the References section of the CPMN book.